The Relationship between Clinician Self-Compassion, Therapeutic Alliance Ruptures, and Self-Reported Competency and Comfort Working with High-Risk Patients

Protocol Paper

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Background

Suicide is a major public health concern as it takes the lives of many each year. Mental health clinicians play a vital role in assessment and intervention. Consistently working with patients at a high-risk for suicide can negatively impact the well-being of clinicians.

Self-compassion is an important trait for clinicians to develop as it can decrease emotional stress (Galili-Weinstock et al., 2024). Additionally, in clinicians, self-compassion can predict greater use of approach coping and reduced use of avoidance coping (Keyes et al., 2022).

It is not uncommon for clinicians to report feeling inadequately prepared when working with high-risk patients (Cramer et al., 2013). More education on working with suicidal patients can meaningfully impact self-reported competency levels.

The therapeutic alliance is an essential component of psychotherapy, especially when it comes to disclosure of suicidal ideation. When the alliance is weak, trust issues or client resistance may emerge (Zarzycka et al., 2021).

Gaps in the literature

Although there has been research on the relationship between clinician self-compassion and the therapeutic alliance, little work has examined whether self-compassion is linked to alliance ruptures specifically. Additionally, current literature does not address the relationship between clinician self-compassion and the alliance when working with high-risk patients specifically.

Little work has been done on whether clinician self-compassion influences clinicians' perceived comfort and competency, especially when working with high-risk patients.

Regarding the relationship between years of experience and self-reported comfort and competency, mixed results have been found.

Study Aims

This study aims to explore the relationship between clinician self-compassion, therapeutic alliance ruptures, and self-reported competency and comfort when working with patients at a high-risk for suicide.

The findings from this study will be used to understand clinicians' experiences when working with high-risk patients to help improve the training and support of clinicians who work with this population.

Research Questions

Research question 1: Comfort and self-reported competency

a) Is clinician self-compassion associated with clinicians' perceived comfort and competency when working with high-risk patients? Given the limited literature on this topic, this research question will remain exploratory, and a hypothesis will not be formulated.

b) Is there a relationship between years of experience and self-reported comfort and competency in clinicians working with high-risk patients? It is expected that clinicians with more years of experience will report higher levels of comfort and competency working with high-risk patients.

Research question 2: Alliance and ruptures

Is there a relationship between clinician self-compassion and the quality of therapeutic alliance ruptures with high-risk patients? Given the limited literature on this topic, this research question will remain exploratory, and a hypothesis will not be formulated.

Planned Methods

Participants

This study will recruit approximately 85-100 mental health clinicians spanning diverse practice settings (psychologists, psychiatrists, social workers, counselors, etc).

Inclusion criteria:

- Current licensure as a mental health clinician
- Actively working with high-risk patients or worked in the past with patients presenting with active suicidal ideation
- Minimum age of 18 years
- Ability to understand and complete English-language questionnaires
- Ability to understand and provide electronic informed consent

Procedures

Participants will access the study through secure Qualtrics links. Participants will review study information and provide informed consent. The survey is expected to take no longer than 30 – 35 minutes.

Measures

The comprehensive self-report survey battery will include:

- Self-compassion Scale Short form (SCS-SF)
- Rupture Resolution Rating System Therapist Version (3RS-SF); adapted
- Suicide Competency Inventory (SCI)
- Suicide Knowledge and Skills Questionnaire (SKSQ); adapted
- Demographic questions

Proposed Analyses

Data analysis for this study will be conducted using SPSS. Cronbach's alpha will be used to assess the internal reliability of the full scale and subscales.

A Pearson correlation will be used to examine the association between clinician self-compassion and comfort and competency, and the relationship between years of experience and self-reported comfort and competency.

A point-biserial correlation will be used to assess the relationship between self-compassion and alliance ruptures.

Implications

Regularly working with patients at high-risk for suicide can create significant professional pressure and emotional strain on clinicians. Therefore, understanding factors that support clinicians' well-being and clinical functioning can help improve the care of patients and clinical outcomes.

Findings from this study can influence how clinicians are trained by emphasizing the importance of self-compassion when working with high-risk patients.

References

Cramer, R. J., Johnson, S. M., McLaughlin, J., Rausch, E. M., & Conroy, M. A. (2013). Suicide risk assessment training for psychology doctoral programs: Core competencies and a framework for training. *Training and Education in Professional Psychology, 7*(1), 1–11. https://doi.org/10.1037/a0031836

Galili-Weinstock, L., Chen, R., Atzil-Slonim, D., Rafaeli, E., & Peri, T. (2020). Enhancement of self compassion in psychotherapy: The role of therapists' interventions. *Psychotherapy Research*, *30*(6), 815-828.

Keyes, J., Yankouskaya, A., & Panourgia, C. (2023). Self-compassion, coping strategies and gender differences in psychology, counselling and psychotherapy practitioners during COVID-19: Lessons learnt. *Counselling and psychotherapy research*, *23*(4), 1052-1062.

Zarzycka, B., Jankowski, T., & Krasiczyńska, B. (2022). Therapeutic relationship and professional burnout in psychotherapists: A structural equation model approach. *Clinical Psychology & Psychotherapy*, *29*(1), 250-259.